

Policy

Missing Inpatients

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

Introduction

Sir Charles Gairdner Osborne Park Health Care Group (SCGOPHCG) has a duty of care to protect and care for patients, particularly those who are vulnerable and/or confused.

Adult inpatients are free to move in and out of an inpatient unit at their discretion. Patients who are not legally detained have the right to leave at any time. Patients should however be asked to inform staff when they wish to leave the ward or hospital.

Risk Statement

Non-compliance with this policy may:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☐
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☐	Staff Safety	☐
Other:	☐		

Definitions

Missing or suspected missing inpatient	Missing or suspected missing inpatient for this policy refers to any admitted patient, - Who has left the immediate area without informing staff, - Who cannot be located despite efforts; and where there is sufficient concern for the patient's health and wellbeing.
Discharge against medical advice (DAMA)	When an inpatient chooses to leave the hospital and/or is removed by their parent/carer/responsible person before the completion of treatment against the advice of the treating team and a decision is made to discharge against the advice of the treating clinician/s. This includes patients leaving: - After receiving advice from a treating clinician - Without receiving advice from a treating clinician - Without advising staff of their intentions

Exclusion to this policy:

- Prisoners in custody; Missing prisoners are the responsibility of the police officers or prison/custodial officers accompanying them.
- Babies: a Code Black Alpha should be initiated in this scenario: See Emergency Management Services HealthPoint page [here](#) for more information.

Policy Principles

This policy provides guidance for SCGOPHCG staff on identifying patients at risk of going missing and the process to be followed when a patient is suspected to be missing.

The policy applies to all staff accepting responsibility for the care of any patient.

Preventative Measures:

The following preventative measures should be considered to minimise the occurrence of patients going missing:

- Advising patients as part of the admission process (through their relatives or carers, if appropriate) to notify staff of their whereabouts if they are departing the ward or hospital.
- Ensuring on admission all patient/ NOK details are up to date including mobile numbers.

Medical Officer assesses patients' cognition/mental capacity if appropriate as part of a risk assessment and document a risk management plan in their patient medical record.

- A patient known to be at risk of wandering behaviour, in addition to the appropriate identification band, will have the form ' Admission Registration Record' completed and placed in the front of the patient's current medical record.

Procedure

In the event of a missing patient staff should try to ascertain the following:

- if any person observed the patient leaving the ward.
- the last time the patient was seen.
- if the patient has any scheduled appointments or treatments that may necessitate the patient to leave the area.
- if relatives or friends were visiting the patient who may have taken the patient out of the ward.
- if the patient has taken their belongings with them.
- If there is documentation in the patient's medical record that indicates they are not to leave/DAMA contact the senior Registrar or Consultant to determine if a missing person report needs to be filed with WA police (WAPOL).

Responsibilities

If the patient's location cannot be accounted for, then:

- Attempt to contact the patient by calling their mobile phone (if number on file)
- If unable to contact inform the Ward Shift Coordinator.

Ward Shift Coordinator:

- arrange a thorough search of the immediate ward environment.
- If the patient is not located during the local search, notify the following staff and provide them a detailed description of the patient and approximate time of leaving the ward:

Business Hours

- Ward Clinical Nurse Specialist – (CNS), SCGOPHCG
- Medical Officer (RMO or Medical Registrar)
- Next of Kin or identified contact person as listed in the patient medical record.
- Security Officer/s (dialling 56)

After Hours

- After Hours Clinical Nurse Manager, OPH
- After Hours Clinical Nurse Specialist, SCGH
- Next of Kin or identified contact person as listed in the patient medical record.
- Security Officer/s (dialling 56)

Ward CNS/ CNM or After-Hours Clinical Nurse Specialist/Manager actions:

- inform Divisional Nurse Co-Director (in hours) or on-call Nurse Executive (after hours)
- in consultation with Security Officer/s consider a search of the hospital grounds.

Security Officer/s:

- Present to the ward area and liaise with Shift Coordinator
- review CCTV footage (as necessary)
- assist in local search
- dispatch two-way radios / emergency mobile phones for communications between search members.
- Inform ward shift coordinator of the outcome

Medical staff:

- Attend ward
- Identify any life-threatening issues that may impact the patient
- Assess if the patients return is imperative
- Inform treating consultant
- Document all decisions in the patient's medical record

1.0 Patient located:

- When the patient is found on the hospital site the patient will be returned to the relevant ward and their identity confirmed
- Notify Security Officer, appropriate Clinical Nurse Specialist / Clinical Nurse Manager / Clinical Midwife Manager, Medical Officer, Police, and the Next of Kin as soon as possible.
- Nursing staff will ensure the following has been documented in the patient's medical record:
 - Time patient returned
 - Name and person returning patient
 - Where patient was found
- The medical officer will examine the patient and document details of the medical examination in the patient's medical record.
- In consultation with the CNS/CNM, medical officer to formulate a plan of care to address safety issues and the implement strategies.
- If the patient is found but declines return to the ward, staff will escalate to the medical team to determine if the patient may DAMA and complete the Discharge Against medical advice (DAMA) form if appropriate. Refer to SCGOPHCG Discharge Against Medical Advice Policy.
- CNS/CNM to register the incident using the online Datix Clinical Incident Management System (CIMS).

2.0 Patient not located within (1) hour:

Ward CNS/CNM or After-Hours Clinical Nurse Specialist/Manager actions:

- Consult with medical officer to determine if a missing person report should now be filed with WAPOL
- Inform the Police on 131444 who will provide a Missing Person Incident Number. They will require the following patient information:
- Any specific health concerns (this will determine with priority of the Police attendance), urgent medical problems, requirements, or medications they need,
- Full name and address,
- Date of birth,
- Gender,
- Description of physical appearance (race, complexion, height, build, hair, and eye colour),
- The clothing the patient was wearing when last seen,
- Any distinctive or distinguishing features,
- The patient's state of mind when last seen,
- The patient's mobility status,
- Whether the patient presents any specific risk and if so the level and nature of that risk; and
- Any other information that may indicate the location of the patient.

Note: Staff are not to leave the hospital grounds to continue with searching for the patient.

Compliance monitoring and evaluation

All incidents related to this policy must be reported via Datix CIMS.

Related Documents

[NMHS Healthcare Record Access, Use and Disclosure policy](#)

[NMHS Clinical Incident Management Policy](#)

[SCGOPHCG Patient Identification Policy](#)

[SCGOPHCG Detention of Patients and Discharge Against Medical Advice](#)

Authorisation & Version Control

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